



Tests you can trust

Name : Mr Vansh Pratap Singh (18Y/M)

Date : 12 Jul 2025

Test Asked : Aarogyam C Pro With Utsh

Report Status: Complete Report



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Controlled
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PROCESSED AT :**Thyrocare**

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NAME : MR VANSH PRATAP SINGH (18Y/M)
REF. BY : S SELF
TEST ASKED : AAROGYAM C PRO WITH UTSH

SAMPLE COLLECTED AT :
(3020213650), ROYAL DIAGNOSTIC PVT
LTD, 17, NEAR ICICI BANK, VIDHUT NAGAR, AJMER
ROAD, JAIPUR, 302021

Report Availability Summary

Note: Please refer to the table below for status of your tests.

🟢 12 Ready

🟡 0 Ready with Cancellation

🔄 0 Processing

🔴 0 Cancelled in Lab

TEST DETAILS

REPORT STATUS

AAROGYAM C PRO WITH UTSH

Ready ✓

CHLORIDE

Ready ✓

HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)

Ready ✓

SODIUM

Ready ✓

TESTOSTERONE

Ready ✓

HBA PROFILE

Ready ✓

HEMOGRAM - 6 PART (DIFF)

Ready ✓

LIVER FUNCTION TESTS

Ready ✓

IRON DEFICIENCY PROFILE

Ready ✓

KIDPRO

Ready ✓

LIPID PROFILE

Ready ✓

T3-T4-USTSH

Ready ✓

VITAMIN D TOTAL AND B12 COMBO

Ready ✓

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ICICI BANK, VIDHUT NAGAR, AJMER ROAD,
JAIPUR, 302021

Summary Report

Tests outside reference range

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
BASOPHILS - ABSOLUTE COUNT	0.01	X 10 ³ / μ L	0.02 - 0.1
EOSINOPHILS	9	%	1-6
EOSINOPHILS - ABSOLUTE COUNT	0.62	X 10 ³ / μ L	0.02 - 0.5
RED CELL DISTRIBUTION WIDTH (RDW-CV)	16.2	%	11.6-14
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	53.7	fL	39-46
LIPID			
HDL / LDL RATIO	0.35	Ratio	> 0.40
LDL CHOLESTEROL - DIRECT	118	mg/dL	< 100
LIVER			
ASPARTATE AMINOTRANSFERASE (SGOT)	52.85	U/L	< 35
SERUM GLOBULIN	3.47	gm/dL	2.5-3.4
RENAL			
BLOOD UREA NITROGEN (BUN)	7.87	mg/dL	7.94 - 20.07
UREA (CALCULATED)	16.84	mg/dL	Adult : 17-43
VITAMIN			
25-OH VITAMIN D (TOTAL)	19.93	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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NAME : MR VANSH PRATAP SINGH (18Y/M)**REF. BY** : S SELF**TEST ASKED** : HBA PROFILE,HEMOGRAM**SAMPLE COLLECTED AT :**

(3020213650),ROYAL DIAGNOSTIC PVT
LTD,17,NEAR ICICI BANK,VIDHUT NAGAR,AJMER
ROAD, JAIPUR,302021

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.1	%

Bio. Ref. Interval. :**As per ADA Guidelines**

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
>=6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	100	mg/dL
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Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values**Please correlate with clinical conditions.****Sample Collected on (SCT)** : 12 Jul 2025 08:04**Sample Received on (SRT)** : 12 Jul 2025 13:08**Report Released on (RRT)** : 12 Jul 2025 15:34**Sample Type** : EDTA Whole Blood**Labcode** : 1207078831/RAJ38**Barcode** : DM294582

Dr Madhavi MD(Path)

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

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REF. BY : S SELF

TEST ASKED : HBA PROFILE,HEMOGRAM

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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	15.1	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	44.5	%	40.0-50.0
Total RBC	HF & EI	4.89	X 10 ⁶ /μL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	91	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	30.9	pg	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	33.9	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	53.7	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	16.2	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	301.5	-	*Refer Note below
MENTZER INDEX	Calculated	18.6	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	6.87	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	45.4	%	40-80
Lymphocytes Percentage	Flow Cytometry	38	%	20-40
Monocytes Percentage	Flow Cytometry	7.1	%	2-10
Eosinophils Percentage	Flow Cytometry	9	%	1-6
Basophils Percentage	Flow Cytometry	0.2	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	3.12	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.61	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.49	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.01	X 10³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.62	X 10³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10 ³ / μL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	215	X 10 ³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	10.4	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	11.9	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	27.9	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.22	%	0.19-0.39

Remarks : Alert!!! RBCs:Mild anisopoikilocytosis. Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

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Report Released on (RRT) : 12 Jul 2025 15:34

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Madhavi

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TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	C.L.I.A	19.93	ng/mL

Bio. Ref. Interval. :

DEFICIENCY : <20 ng/ml || INSUFFICIENCY : 20-<30 ng/ml

SUFFICIENCY : 30-100 ng/ml || TOXICITY : >100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml.

Kit Validation Reference: Holick MF. Vitamin D Deficiency. N Engl J Med. 2007;357:266-81.

Method : Fully Automated Chemi Luminescent Immuno Assay

VITAMIN B-12	C.L.I.A	221	pg/mL
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Bio. Ref. Interval. :

Normal : 211 - 911 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:

Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569-73.

Method : COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

Please correlate with clinical conditions.

Sample Collected on (SCT) : 12 Jul 2025 08:04

Sample Received on (SRT) : 12 Jul 2025 13:10

Report Released on (RRT) : 12 Jul 2025 15:59

Sample Type : SERUM

Labcode : 1207079065/RAJ38

Barcode : DQ863396



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TEST NAME	TECHNOLOGY	VALUE	UNITS
TESTOSTERONE	C.L.I.A	674.02	ng/dL

Bio. Ref. Interval. :-

Adult Male

21 - 49 Yrs : 164.94 - 753.38 || 50 - 89 Yrs : 86.49 - 788.22

Adult Female

Pre-Menopause : 12.09 - 59.46 || Post-Menopause: < 7.00 - 48.93

Boys

2-10 Years : < 7.00 - 25.91

11 Years : < 7.00 - 341.53

12 Years : < 7.00 - 562.59

13 Years : 9.34 - 562.93

14 Years : 23.28 - 742.46

15 Years : 144.15 - 841.44

16-21 Years : 118.22 - 948.56

Girls

2-10 Years : < 7.00 - 108.30

11-15 Years : < 7.00 - 48.40

16-21 Years : 17.55 - 50.41

Clinical Significance: Clinical evaluation of serum testosterone, along with serum LH, assists in evaluation of Hypogonadal males.

Major causes of lowered testosterone in males include Hypogonadotropic hypogonadism, testicular failure Hyperprolactinemia, Hypopituitarism some types of liver and kidney diseases and critical illness.

Specifications: Precision: Intra assay (%CV): 8.5 %, Inter assay (%CV): 12.6%; Sensitivity: 7 ng/dL.

Kit Validation Reference: Kicklighter EJ, Norman RJ. The gonads. In: Kaplan LA, Pesce AJ, eds. Clinical Chemistry: Theory, Analysis, Correlation. 2nd ed. St. Louis: CV Mosby; 1989:657-662.

Please correlate with clinical conditions.**Method:-** COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY**Sample Collected on (SCT)** : 12 Jul 2025 08:04**Sample Received on (SRT)** : 12 Jul 2025 13:10**Report Released on (RRT)** : 12 Jul 2025 15:59**Sample Type** : SERUM**Labcode** : 1207079065/RAJ38 Dr Madhavi MD(Path)**Barcode** : DQ863396

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	< 0.1	mg/L

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
> 3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection, active arthritis or concurrent illness.

Clinical significance:

High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:

1. Clinical management of laboratory data in medical practice 2003-2004, 207(2003).
2. Tietz : Textbook of Clinical Chemistry and Molecular diagnostics : Second edition : Chapter 47: Page no. 1507- 1508.

Please correlate with clinical conditions.**Method:-** FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 Method : Ferrozine method without deproteinization	PHOTOMETRY	103.63	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl Method : Spectrophotometric Assay	PHOTOMETRY	368.73	µg/dL
% TRANSFERRIN SATURATION Bio. Ref. Interval. : 13 - 45 Method : Derived from IRON and TIBC values	CALCULATED	28.1	%
UNSAT. IRON-BINDING CAPACITY (UIBC) Bio. Ref. Interval. : 162 - 368 Method : SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	265.1	µg/dL

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	168	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	42	mg/dL	40-60
HDL / LDL RATIO	CALCULATED	0.35	Ratio	> 0.40
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	118	mg/dL	< 100
TRIG / HDL RATIO	CALCULATED	1.83	Ratio	< 3.12
TRIGLYCERIDES	PHOTOMETRY	76	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	4	Ratio	3 - 5
LDL / HDL RATIO	CALCULATED	2.8	Ratio	1.5-3.5
NON-HDL CHOLESTEROL	CALCULATED	126.47	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	15.25	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase

HCHO - Direct Enzymatic Colorimetric

HD/LD - Derived from HDL and LDL values.

LDL - Direct Measure

TRI/H - Derived from TRIG and HDL Values

TRIG - Enzymatic, End Point

TC/H - Derived from serum Cholesterol and Hdl values

LDL/ - Derived from serum HDL and LDL Values

NHDL - Derived from serum Cholesterol and HDL values

VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	96.5	U/L	58-331
BILIRUBIN - TOTAL	PHOTOMETRY	0.61	mg/dL	0.3-1.2
BILIRUBIN - DIRECT	PHOTOMETRY	0.11	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.5	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	20.97	U/L	< 55
SGOT / SGPT RATIO	CALCULATED	1.18	Ratio	< 2
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	52.85	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	44.99	U/L	< 45
PROTEIN - TOTAL	PHOTOMETRY	8.07	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.6	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.47	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.33	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
OT/PT - Derived from SGOT and SGPT values.
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
PROT - Biuret Method
SALB - Albumin Bcg¹ method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST ASKED : AAROGYAM C PRO WITH UTSH

SAMPLE COLLECTED AT :

(3020213650), ROYAL DIAGNOSTIC PVT LTD, 17, NEAR
ICICI BANK, VIDHUT NAGAR, AJMER ROAD,
JAIPUR, 302021

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	7.87	mg/dL	7.94 - 20.07
UREA (CALCULATED)	CALCULATED	16.84	mg/dL	Adult : 17-43
CREATININE - SERUM	PHOTOMETRY	0.77	mg/dL	0.72-1.18
UREA / SR.CREATININE RATIO	CALCULATED	21.87	Ratio	< 52
BUN / SR.CREATININE RATIO	CALCULATED	10.22	Ratio	9:1-23:1
CALCIUM	PHOTOMETRY	10	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	5.49	mg/dL	4.2 - 7.3
SODIUM	I.S.E - INDIRECT	139.8	mmol/L	136 - 145
CHLORIDE	I.S.E - INDIRECT	102.16	mmol/L	98 - 107

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.

UREAC - Derived from BUN Value.

SCRE - Creatinine Enzymatic Method

UR/CR - Derived from UREA and Sr.Creatinine values.

B/CR - Derived from serum Bun and Creatinine values

CALC - Arsenazo III Method, End Point.

URIC - Uricase / Peroxidase Method

SOD - ION SELECTIVE ELECTRODE - INDIRECT

CHL - ION SELECTIVE ELECTRODE - INDIRECT

Sample Collected on (SCT) : 12 Jul 2025 08:04

Sample Received on (SRT) : 12 Jul 2025 13:10

Report Released on (RRT) : 12 Jul 2025 15:59

Sample Type : SERUM

Labcode : 1207079065/RAJ38

Barcode : DQ863396

Madhavi

Dr Madhavi MD(Path)

PROCESSED AT :**Thyrocare**

1st Floor, Showroom No.4 to 11 A,
Unique Destination, Tonk Road,
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NAME : MR VANSH PRATAP SINGH (18Y/M)**REF. BY** : S SELF**TEST ASKED** : AAROGYAM C PRO WITH UTSH**SAMPLE COLLECTED AT :**

(3020213650), ROYAL DIAGNOSTIC PVT LTD, 17, NEAR
ICICI BANK, VIDHUT NAGAR, AJMER ROAD,
JAIPUR, 302021

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	74	ng/dL	60-200
TOTAL THYROXINE (T4)	C.L.I.A	4.8	µg/dL	4.5-12
TSH - ULTRASENSITIVE	C.L.I.A	4.276	µIU/mL	0.55-4.78

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay

T4 - Competitive Chemi Luminescent Immuno Assay

USTSH - Third Generation Ultrasensitive Chemi Luminescent Immuno Assay

References :

1. Elmlinger MW, Kuhnel W, Lambrecht HG, et al. Reference intervals from birth to adulthood for serum thyroxine, T3, free T3, Free T4, TBG and TSH. Clin Chem lab med. 2001; 39:973
2. Edward CC, Carlo B. Paediatric Reference Intervals. 8th edition. 2021

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Sample Collected on (SCT) : 12 Jul 2025 08:04**Sample Received on (SRT)** : 12 Jul 2025 13:10**Report Released on (RRT)** : 12 Jul 2025 15:59**Sample Type** : SERUM**Labcode** : 1207079065/RAJ38 Dr Madhavi MD(Path)**Barcode** : DQ863396

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LTD, 17, NEAR ICICI BANK, VIDHUT NAGAR, AJMER
ROAD, JAIPUR, 302021

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	133	mL/min/1.73 m ²

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.**Method:-** 2021 CKD EPI Creatinine Equation

~~ End of report ~~

Sample Collected on (SCT) : 12 Jul 2025 08:04**Sample Received on (SRT)** : 12 Jul 2025 13:10**Report Released on (RRT)** : 12 Jul 2025 15:59**Sample Type** : SERUM**Labcode** : 1207079065/RAJ38 Dr Madhavi MD(Path)**Barcode** : DQ863396

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Scan QR code to verify authenticity of reported results; active for 30 days from release time.

CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
- ✓ Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- ✓ Thyrocare Discovery video link :- <https://youtu.be/nbdYeRgYyQc>

EXPLANATIONS

- ✓ Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- ✓ **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- ✓ **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ Testing or retesting should be done in accredited laboratories.
- ✓ For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**

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*T&C Apply, #As on 5th December 2024, *As per a survey on doctors' perception of laboratory diagnostics (IJARIIT,2023)